Rutland Regional Medical Center
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January 30, 2026

Ms. Kathryn O'Neill
Director of Prescription Drug Pricing -Green Mountain Care Board
GMCB.PrescriptionDrugAffordabilityTeam@vermont.gov

Sent via email

Re: Rutland Regional's 2026 Hospital 340B Reporting Requirements

Dear Ms. O'Neill:

Please accept this letter which will serve as Rutland Regional Medical Center's (RRMC) submission to meet the Vermont Hospital 340B Reporting as required by Act 55 of 2025. We appreciate the Green Mountain Care Board's interest in 340B transparency and look to partner with The Board on this matter so we can show how vital the 340B Program is for us.

Narrative Reporting Requirement #1: A description of the community health initiatives or programs that received funding from your hospital's 340B program participation, including how these investments are intended to improve access to care and quality of services. Specify the aggregate amount invested toward all such programs.

In FY2025, 340B program revenue supported hospital investments, initiatives and programs intended to improve access to care, enhance quality of services, and reduce financial barriers for patients and communities served.

1. **West Ridge Center**

340B program revenue supports our West Ridge Center opioid treatment clinic, because available state funding covers only 18% of costs associated with operating a high-quality, comprehensive program. West Ridge is intentionally designed with a biopsychosocial model of care, where in addition to providing medications to treat opioid use disorder (OUD), we provide enhanced counseling and robust wraparound supports such as therapy, case management, care coordination, and communicable disease screening/education. In the last year, the clinic strengthened interdisciplinary team-based care and transitioned to an Open Access model (Monday–Friday, 6–11am) to improve access and reduce wait times, enabling same-day assessment and connection to supportive resources when patients are ready to seek help. The clinic also expanded integrated services through a full time nurse practitioner, who helps provide primary care provider services for comorbid conditions commonly associated with OUD, such as wound care, access to hepatitis C treatment initiation through West Ridge and HIV prevention options with Pre-Exposure Prophylaxis (PrEP). West Ridge has also been recognized by National Committee for Quality Assurance as a patient-centered specialty practice, reflecting its focus on coordinated, integrated care.

2. **Breast Health / Mammography modernization**

340B program revenue supported a $1,109,082 capital investment to upgrade our mammography equipment with an installation of two new mammography rooms, including installation of Hologic 3D digital breast tomosynthesis mammography systems, iCAD AI licenses, and biopsy equipment. This

investment expands community access to breast cancer screening and diagnostic services by increasing imaging capacity, reducing wait times, and ensuring patients can receive advanced breast imaging services locally. This upgrade improves quality by enhancing image detail and detection sensitivity, which supports earlier and more confident detection and characterization of abnormalities, and by improving workflow consistency through AI-supported review of 3D mammography studies.

3. **Endoscopy equipment modernization**

340B program revenue supported a $1,144,684 investment to replace and modernize the Endoscopy Department's endoscopy towers and scopes with the FujiFilm Eluxeo platform. This investment enhances quality of care by improving visualization during endoscopic procedures, supporting more accurate and confident identification and characterization of clinical findings. Additionally, this upgrade supports implementation of the Provation Endoscopy EMR (which went live on December 9, 2025) by providing a modernized technical and workflow foundation for standardized documentation and more efficient image/report management, further strengthening continuity and quality of care for patients in the community.

4. **Financial Assistance Program**

340B program revenue also helps sustain the hospital's Financial Assistance Program, structured to remove cost barriers for patients with limited ability to pay. The program provides 100% financial assistance (free care) for eligible patients up to 400% of the Federal Poverty Level (FPL), and a 50% discount for eligible patients between 401%–500% FPL. Through support from the 340B Program, RRMC is able to maintain the most generous financial assistance program in the state of Vermont, ensuring that cost does not prevent community members from seeking needed care. Total financial assistance is $11.7 million, underscoring the hospital's commitment to community access and affordability.

Narrative Reporting Requirement #2: A description of the hospital's internal review and oversight of its participation in the 340B program in compliance with the U.S. Department of Health and Human Services, Health Resources and Services Administration's 340B program rules and guidance.

The hospital maintains a structured internal oversight framework for its 340B program intended to align with HRSA Office of Pharmacy Affairs (OPA) program requirements and guidance. The hospital's 340B compliance approach emphasizes auditable recordkeeping and controls designed to protect against key compliance risks including diversion, Medicaid duplicate discounts, and compliance with the group purchasing organization (GPO) prohibition, with appropriate record retention.

To reinforce independence and objectivity, the hospital engages an external auditor to provide specialized 340B guidance and to perform an annual independent external 340B audit. The most recent independent review included evaluation of eligibility controls, diversion and duplicate-discount prevention mechanisms, OPA registration accuracy, and review of key policies and procedures, along with sample-based testing. Overall, the review did not identify concerns related to eligibility, diversion, or duplicate discounts and provided targeted recommendations to further enhance program controls and readiness, including strengthening documentation and refining aspects of internal monitoring and record maintenance. The results of our yearly independent audits are presented to the Audit and Compliance Committee of our Board.

Narrative Reporting Requirement #3: A description of how the aggregated payment amount is calculated for payments made for all other internal expenses related to administering the 340B program during fiscal year 2025.

Internal expenses include those directly billed services for third party software support, auditing, management and oversight of the 340B program. In addition, internal expenses reflect administrative and operational costs of RRMC leadership, pharmacy, retail pharmacy, and finance and accounting staff. These costs are documented and are aggregated under the Payor (Uncategorized) section in our Adaptive submission.

Lastly, as noted in RRMC's 340B Guidance Public Comment Response (November 19, 2025), payer reimbursement is not consistently available at the individual drug-line level, so portions of RRMC's drug reimbursement analysis required additional data mining and estimation. Specifically, commercial payers in Vermont do not consistently issue separate payment specific to a given medication. Instead, medications (including high-cost infusion drugs) may be reimbursed as part of a bundled or lump-sum payment to the hospital. These payments are not broken out by medication and do not include a drug-level allocation or percentage attributable to a specific drug. As a result, drug-associated payment amounts reflected in the analysis may be estimated. Where bundled payments applied, Rutland Regional Medical Center complied with the calculation methodology required by the reporting guidance. RRMC will continue to refine this process for upcoming reporting cycles to improve data completeness and accuracy.

Please do not hesitate to contact us should you require any additional information or further discussion.

Sincerely,

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cc: Health Care Advocate (hcapolicyteam@vtlegalaid.org)